

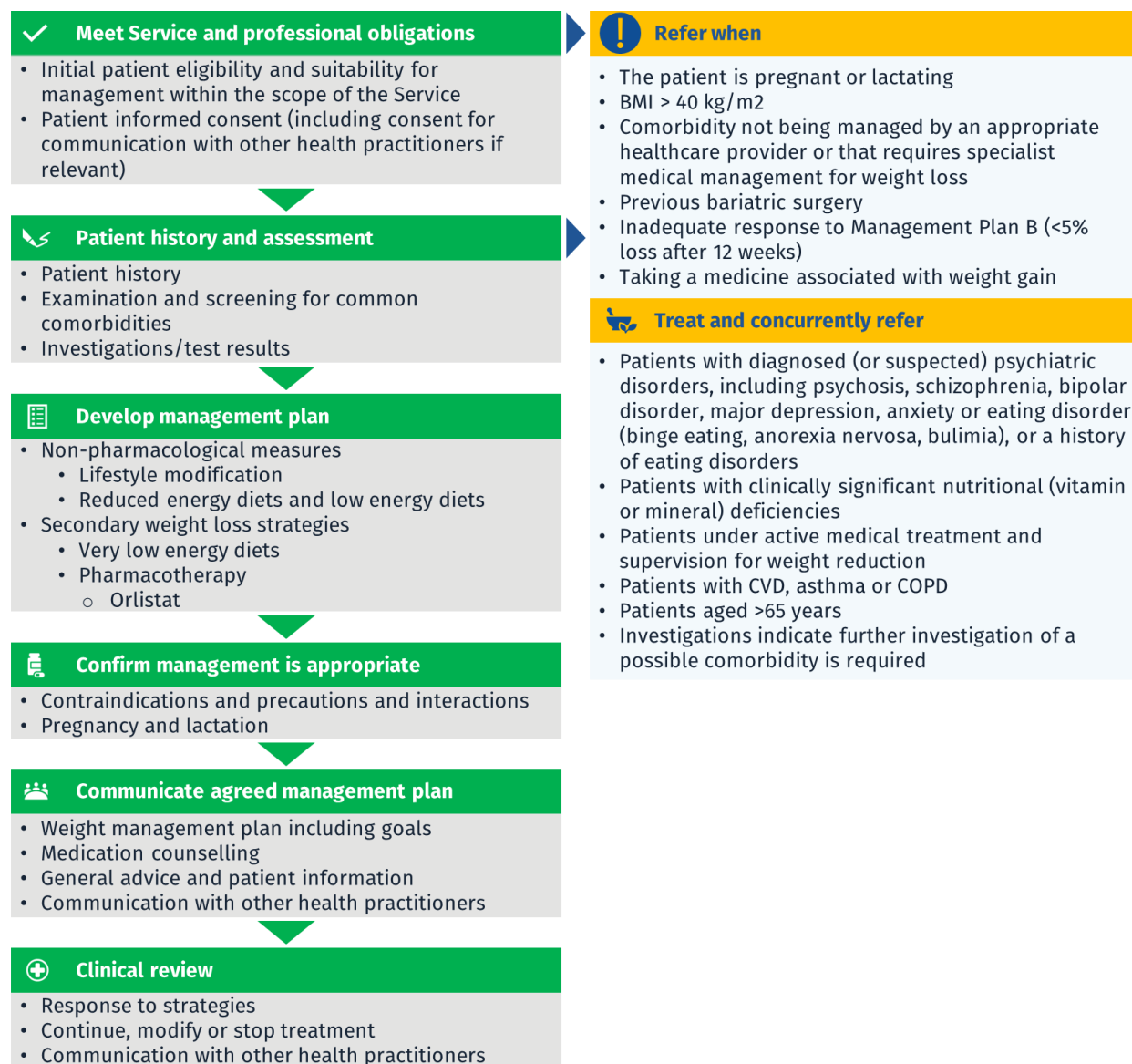
Queensland Community Pharmacy

Management for Overweight and Obesity - Clinical Practice Guideline

Service restricted to patients aged 18 years or over.

Ineligibility for treatment as part of a prescribing service does not preclude a patient from accessing services provided as part of usual pharmacy care.

Guideline overview



When applying the information contained within this clinical practice guideline, pharmacists are advised to exercise professional discretion and judgement. The clinical practice guideline does not override the responsibility of the pharmacist to make decisions appropriate to the circumstances of the individual, in consultation with the patient and/or their parent/caregiver.



Refer when

Refer the patient to an appropriate healthcare provider in the following situations:

- Patient is pregnant or lactating
- BMI > 40 kg/m²
- Comorbidity not being managed by an appropriate healthcare provider* or that requires specialist management for weight loss
- Previous bariatric surgery
- Taking a medicine associated with weight gain
- Inadequate response to Management Plan B (<5% loss after 12 weeks).

Treat (if clinically appropriate) and concurrently refer

Provide initial treatment and concurrently refer to an appropriate healthcare provider for collaborative management in the following situations:

- Patients with diagnosed (or suspected) psychiatric disorders, including psychosis, schizophrenia, bipolar disorder, major depression, anxiety or eating disorder (binge eating, anorexia nervosa, bulimia), or a history of eating disorders
- Patients with clinically significant nutritional (vitamin or mineral) deficiencies
- Patients under active medical treatment and supervision for weight reduction
- Patients with CVD, asthma or COPD*
- Patients aged >65 years
- Findings indicate further investigation of a possible comorbidity is required*.



Reminder

*Patients with hypertension, dyslipidaemia, mild COPD and mild to moderate asthma may be suitable for management as part of the Queensland Community Pharmacy Chronic Conditions Management Pilot – refer to the respective clinical protocols for information and eligibility criteria if in a participating pharmacy.

Key points

- Human behaviour, genetics, environment and lifestyle all play a role in managing body weight ^(1, 2).
- The social determinants of health influence a patient's risk of being overweight and obese, of which social inequality and disadvantage contribute to unfair and avoidable differences in health outcomes within groups in society ⁽³⁾.
- Excess weight (overweight and obesity), generally defined in adults as a body mass index (BMI) ≥ 25 kg/m², is a risk factor for metabolic and chronic conditions, including type 2 diabetes, cardiovascular disease (CVD) and some cancers, leading to higher rates of premature morbidity and mortality ^(4, 5).
- Body mass index (BMI) is an internationally recognised standard for classifying overweight and obesity in adults, particularly in a population health context ^(4, 6). BMI does not directly

measure body fat and may be insensitive to body composition, particularly in older people and very muscular people/athletes; however, it is strongly correlated to other direct measures of body fat and obesity-related comorbidities (CVD) ^(6, 7).

- Excess body weight for height is associated with osteoarthritis, poor mental health and conditions affecting the reproductive and gastrointestinal systems ⁽⁷⁾.
- While many people can achieve short-term weight loss, sustained weight loss/maintenance over the long-term (over 2 years) is required for cardiovascular benefits ⁽⁷⁾.
- Effective and long-term management of overweight and obesity requires a multidisciplinary team care approach tailored to the individual ^(4, 8-10).
- Lifestyle modification is the foundation of weight management and for some patients, lifestyle modification alone may be sufficient to achieve weight loss; evidence suggests that interventions that address nutrition, physical activity and psychological approaches to behavioural change are more effective for weight loss than single interventions ^(2, 7). Social prescribing to local non-clinical services may support lifestyle modification.
- Additional strategies may include behavioural and psychological therapy, pharmacotherapy and bariatric surgery ⁽¹¹⁾.
- Weight and weight loss are sensitive issues for many people and most individuals with overweight and obesity will have attempted multiple weight loss interventions ⁽¹²⁾. It is important to communicate and provide care to patients in a way that is person-centred, culturally sensitive, non-judgmental, and private ⁽¹³⁾.

Patient history and assessment

Each patient's needs regarding weight management and suitability for pharmacist management should be assessed on a case-by-case basis.

Patient history

Obtain sufficient information from the patient to assess the safety and appropriateness of any recommendations and medicines.

The patient history should consider:

- age
- pregnancy and lactation status (if applicable)
- weight history including highest and lowest weights (and when)
- previous weight loss and strategies used, including the degree of success and levels of medical/health practitioner support (e.g., previous drug therapy, very-low energy diets (VLED), exercise)
- recent relevant biochemical markers (see [Investigations](#))
- medical conditions and comorbidities, including weight and non-weight related
- known, suspected or previous eating disorder (see [Appendix 1](#))
- surgical history, including any bariatric surgery
- current and recently ceased treatments (including prescribed medicines, over-the-counter medicines, vitamins, herbs, powders (like protein powders), other supplements)

- drug allergies/adverse drug reactions
- alcohol and drug history
- smoking status
- diet history (usual intake over the last 6-month period) and fluid intake. Use objective measures (e.g. food diary, food tracking app, measure portions)
- current level of physical activity (structured and incidental activity)
- Other factors impacting upon successful weight management:
 - cooking/shopping skills
 - health and nutritional literacy
 - cooking/shopping responsibilities, including the number of people in the household
 - stage of readiness to change
 - patient support networks.



Reminder

Pharmacists can access a range of clinical information in a patient's My Health Record, including details about current and past medication history, allergies and current medical conditions.

Examination and screening

All patients should undergo basic screening and examination for common weight-related comorbidities and eating disorders. Information to guide screening has been provided in [Appendix 1](#). However, it is also recommended that the patient be comprehensively screened, either as part of the [Queensland Community Pharmacy Pilot Cardiovascular Disease \(CVD\) Risk Reduction Program](#) (if eligible) or by referred to their usual healthcare provider.

If the patient is enrolled in the Queensland Community Pharmacy Pilot CVD Risk Reduction Program, examination does not need to be repeated (relevant screening that has not previously been undertaken should still occur).

Investigations

Recommended investigations are outlined in Table 2.

If available, recent test results may be used (e.g., provided by a medical practitioner with referral or available on My Health Record) if there have been no changes to the patient's medication or health status and the test was conducted within the past 12 months.

Consider investigation for secondary cause of weight change if clinically suspected based on patients presenting signs and symptoms, e.g. thyroid dysfunction.

Table 2. Recommended investigations/tests (based on personal and clinical circumstances)	
Glycated haemoglobin (HbA1c) or fasting blood glucose (FBG) ^{(14) NB1}	
Testing requirements	Referral requirements
- People without a previous diagnosis of diabetes AND - no HbA1c or FBG test results from the previous 3 years AND - at least one of the following criteria are met - AUSDRISK score of ≥ 12 - Aboriginal and Torres Strait Islander people aged ≥ 18 years - Pacific Islander, Indian sub-continent, Southern European, North African, Latin American, Middle Eastern or Asian background - history of gestational diabetes - polycystic ovary syndrome - taking antipsychotic medicines - aged ≥ 40 and overweight or obese - aged 18–40, overweight or obese and hypertension - aged 18–40, overweight or obese and clinical evidence of insulin resistance - history of a cardiovascular event - history of impaired glucose tolerance (IGT) or impaired fasting glucose (IFG) - has a first degree relative with type 2 diabetes 1 or more classical symptoms of diabetes (weight loss, polyuria, polydipsia, blurred vision).	Concurrent referral to an appropriate healthcare provider is required for further investigation of diabetes when: - HbA1c of $\geq 6.5\%$ (48 mmol/mol) - Fasting blood glucose ≥ 7.0 mmol/L. Immediate/emergency referral: HbA1c $>10\%$ (86 mmol/mol) or any blood glucose level ≥ 20.0 mmol/L is considered to be severe hyperglycaemia and is a medical emergency in patients who are unwell (signs of ketosis, dehydration, vomiting, signs or symptoms of underlying infection, confusion and/or delirium/altered level of consciousness) ⁽¹⁵⁾.
Serum lipids (fasting) (including LDL-C, HDL-C and triglycerides) ⁽¹⁶⁻¹⁸⁾	
Testing requirements	Referral requirements
- If lipid testing has not been performed in the previous 5 years AND - Aboriginal and Torres Strait Islander people aged ≥ 30 years OR	Concurrent referral to an appropriate healthcare provider for further investigation of dyslipidaemia when: total cholesterol (TC): ≥ 5.5 mmol/L (or >4.0 mmol/L for people on lipid modifying drug therapy)

○ non-Aboriginal or Torres Strait Islander people aged ≥45 years.	• LDL-C: ≥2.0 mmol/L • HDL-C ≤1.0 mmol/L • Triglycerides: ≥2.0 mmol/L.
BP measurement ⁽¹⁹⁾	
Testing requirements	Referral requirements
All patients: • BP measurement should be conducted in accordance with the National Heart Foundation of Australia: Guidelines for the diagnosis and management of hypertension in adults 2016 ⁽¹⁹⁾.	Concurrent referral to an appropriate healthcare provider for further investigation of hypertension and CVD: • BP ≥140/90 mmHg. Immediate/ emergency referral: Systolic BP ≥180 and/or diastolic BP ≥110 mmHg is considered to be severe hypertension and is a medical emergency in patients with signs or symptoms of a hypertensive urgency (headache or dizziness).
Thyroid stimulating hormone (TSH) ⁽²⁰⁻²²⁾	
Testing requirements	Referral requirements
• If clinically suspected based on patient signs and symptoms. • Weight gain may be a symptom of hypothyroidism.	Concurrent referral to an appropriate healthcare provider for further investigation of thyroid function if: • TSH >4 mIU/L.

NB1: Pharmacists should consider the limitations of HbA1c tests and factors that may cause hyperglycaemia and misleading test results, as per the [Therapeutic Guidelines: Tests to diagnose diabetes](#), and the [RACGP: Management of type 2 diabetes](#) ^(14, 15).

Develop management plan

The management of overweight and obesity, in accordance with the [Australian Obesity Management Algorithm](#) is centred around lifestyle interventions (initial weight-loss strategies) that are tailored to relevant factors for the individual ⁽¹²⁾.

Management may include more intensive interventions (secondary weight-loss strategies) if initial weight loss strategies have not been effective (if clinically indicated and appropriate) ⁽¹²⁾.

Lifestyle interventions are indicated for all patients with overweight or obesity. The [Australian Obesity Management Algorithm](#) provides guidance on the use of pharmacotherapy for patients with BMI ≥30 kg/m² ⁽¹²⁾. Orlistat is indicated for patients with BMI ≥27 kg/m² and other cardiovascular risk factors ⁽²³⁾.

Pharmacists should also consider referral to, and collaboration with, other members of the multidisciplinary health team, depending on the patient's needs, location and financial means.

Management Plan A (Initial weight loss strategies)

Lifestyle modification (for relevant factors):

- Counselling and education for lifestyle modification^{NB1} as per the 5As framework detailed in the [RACGP Smoking, nutrition, alcohol and physical activity \(SNAP\) guide](#) ⁽²⁴⁾ the

[Handbook of Non-Drug Interventions \(HANDI\)](#) ⁽²⁵⁾ and the [Australian Obesity Management Algorithm](#) ⁽¹²⁾.

- It is recommended that consenting patients are referred to an appropriate healthcare provider for physical activity and psychosocial interventions, where required.

Reduced energy diets (RED) and low energy diets (LED):

- In accordance with the [Australian Obesity Management Algorithm](#) ⁽¹²⁾.
- It is recommended that consenting patients are referred to a dietitian for specialised and individualised advice and a tailored eating plan.

Management Plan B (Secondary weight loss strategies)

Management Plan B can be considered for individuals who have not adequately responded to initial weight loss strategies as described in Management Plan A, including where these may have been attempted outside of this Service as part of another structured weight loss program ⁽¹²⁾.

Pharmacists must ensure that they are prescribing medicines according to the specified sections of the current online version of the Therapeutic Guidelines.

Very low energy diets (VLEDs)^{NB2:}

- For individuals who have not adequately responded to a RED or LED, in accordance with individual product instructions and indications, and the [Australian Obesity Management Algorithm](#) ⁽¹²⁾.
- It is recommended that consenting patients are referred to a dietitian for specialised and individualised advice and a tailored eating plan.

Pharmacotherapy:

- For patients following Management Plan A showing less than a 5% decrease in weight after 3 months **AND** with a BMI >27 kg/m² (if other cardiovascular risk factors are present) or BMI >30 kg/m² (no other CVD risk factors or comorbidities):
 - prescribe Orlistat, in accordance with the [Australian Medicines Handbook: Orlistat](#) ^(23, 26). Liver function should be monitored. If Orlistat is unsuitable, refer to an appropriate healthcare provider for other pharmacological management.

NB1:

- For smoking cessation, refer to the [Queensland Community Pharmacy Smoking Cessation Clinical Practice Guideline](#) for management.
- For recommendations of alcohol consumption, refer to the [Australian guidelines to reduce health risks from drinking alcohol](#) ⁽²⁷⁾.
- For advice regarding physical activity focusing on increasing energy expenditure, refer to the [Physical activity and exercise guidelines for all Australians](#) ⁽²⁸⁾.
- For general diet/nutrition advice focusing on reducing energy intake and optimising diet quality as per the [Australian Dietary Guidelines](#) ⁽²⁹⁾.

NB2: VLEDs are recommended to be used in collaboration with a dietitian and medical practitioner for periodic monitoring of biochemistry (electrolytes, creatinine, liver function tests, fasting glucose, lipids, uric acid) and haematology (full blood count, iron studies).

Setting appropriate weight management goals

Weight management is challenging for most people. Weight loss prompts compensatory physiological responses that oppose further weight loss and promote weight gain.

Weight loss goals should be set in collaboration with the patient and reviewed regularly. Targets and goals should be patient-centred and individualised, and encourage sustainable, weight loss that can be maintained over the long-term ^(7, 12).

A weight loss of between 5% and 10% is achievable for most overweight adults and will provide cardiovascular and other health benefits.

Encourage patients to focus on their achievements and progress, rather than what they have not achieved ^(30, 31). In some cases, it may be more useful to encourage them to focus on preventing weight gain than trying to lose weight ⁽³²⁾.

Confirm management is appropriate

Consult the *Australian Medicines Handbook*, the Australian Obesity Management Algorithm and other relevant references to confirm that management is appropriate, including for:

- contraindications and precautions
- drug interactions
- pregnancy and lactation.

Communicate agreed management plan

Comprehensive advice and counselling (including supporting written information if required), should be provided to the patient regarding the weight management plan. Relevant information may include:

- instructions for individual product and medicine use
- non-pharmacological measures
- how to manage adverse effects, including serious adverse effects requiring medical care
- when to return for follow up.

A Weight Management Plan template has been provided in Appendix 2 for guidance.

It is the pharmacist's responsibility to ensure the suitability and accuracy of any resources and information provided to patients (and/or parents/caregivers), and compliance with all copyright conditions.

The agreed management plan should be shared with members of the patient's multidisciplinary healthcare team, with the patient's consent.



Patient resources

- Queensland Government websites:
 - [Staying healthy](#)
 - [Healthier Happier](#)
- Queensland Health: [Nutrition Education Materials](#)
- Baker Heart and Diabetes Institute: [Health Hub Fact sheets](#)

- Australian Government: [Eat for health calculators](#)

Clinical review

Follow up and clinical review is an important component of overweight and obesity management. Clinical review with the pharmacist (or the patient's chosen healthcare provider) should occur in line with recommendations in the [Australian Obesity Management Algorithm](#).

Clinical review on a monthly basis is recommended to monitor progress towards the weight loss target, efficacy of lifestyle modification, and to monitor for adverse effects.

Clinical review appointments may include:

- weight and waist circumference measurements, and BMI calculation
- blood pressure monitoring
- brief advice and reinforcement of lifestyle modification
- review of adverse effects.

Pharmacists may also consider repeating laboratory testing (where indicated).

For patients continuing therapy, follow-up should be individualised to the patient's clinical presentation, response to treatment, and therapeutic goals.

Pharmacists should prescribe a quantity of medicine (including repeats), that aligns with the expected duration of therapy, ensuring sufficient supply until the patient's next review.



Pharmacist resources

- Therapeutic Guidelines:
 - [Cardiovascular](#)
 - [Diabetes](#)
 - [Respiratory](#)
 - [Psychotropic](#)
- Australian Medicines Handbook:
 - [Orlistat](#)
 - [Liraglutide](#)
 - [Naltrexone with bupropion](#)
 - [Phentermine](#)
 - [Semaglutide](#)
 - [Tirzepatide](#)
- Australian Pharmaceutical Formulary and Handbook (APF): [Treatment guideline for pharmacists \(Weight management\)](#)
- [The Australian Obesity Management Algorithm: A simple tool to guide the management of obesity in primary care](#)
- Royal Australian College of General Practitioners:
 - [Smoking, nutrition, alcohol, physical activity \(SNAP\): A population health guide to behavioural risk factors in general practice](#)

- [Guidelines for preventive activities in general practice](#)
- [Handbook of non-drug interventions \(HANDI\)](#)
- National Health and Medical Research Council:
 - [Australian Dietary Guidelines](#)
 - [Australian Guidelines to reduce health risks from drinking alcohol](#)
- Australian Government Department of Health and Aged Care: [Physical activity and exercise guidelines for all Australians](#)
- Australian CVD risk calculator: [CVDCheck](#)
- StatPearls: [Obesity and Comorbid Conditions](#)
- Australian Family Physician:
 - [Pharmacotherapy for obesity](#)
 - [Obesity and weight management at menopause](#) (includes detailed information on VLEDs)
 - [Bariatric-metabolic surgery: A guide for the primary care physician](#)
 - [The bariatric surgery patient: nutrition considerations](#)
- Australian Journal of General Practice: [Bariatric surgery](#)
- [The Australian and New Zealand Obesity Society](#)
- Australian peak body for obesity: [The Obesity Collective](#)
- Diabetes, Metabolic syndrome and Obesity: [Medications that cause weight gain and alternatives in Canada: A narrative review](#)
- Butterfly Foundation – [Eating disorders and body image support](#)
- [InsideOut Institute for Eating Disorders](#)

Appendix 1 – Screening for weight related co-morbidities

Co-morbidity screening (based on personal and clinical circumstances)			
Obstructive Sleep Apnoea (12, 33, 34)			
Screening using the STOP-BANG questionnaire			Referral requirements
STOP	YES (=1)	NO (=0)	Concurrent referral to an appropriate healthcare provider for further investigation is recommended for all patients suspected of having sleep apnoea or another weight-related respiratory condition, for example: - Patients with at least one risk factor or comorbidity + multiple symptoms of sleep apnoea - A score of ≥3 on the STOP-Bang Questionnaire.
Do you SNORE loudly (louder than talking or loud enough to be heard through closed doors)?			
Do you often feel TIRED , fatigued, or sleepy during daytime?			
Has anyone OBSERVED you stop breathing during your sleep?			
Do you have or are you being treated for high blood PRESSURE ?			
BANG			
BMI more than 35kg/m²?			
AGE over 50 years old?			
NECK circumference >16 inches (40cm)?			
GENDER : Male?			
Total Score			
BMI and waist circumference (2, 4, 7, 35)			
Requirements		Referral requirements	
All patients: - Measurement of BMI and waist circumference at commencement and regular reviews (recommended monthly) is required to inform the management plan and assist with patient education regarding risk factors for obesity comorbidities. - A larger waist circumference indicates visceral fat deposits and central obesity: - Males with a waist circumference ≥94 cm are at increased risk of CVD and other comorbidities (≥102 cm = a greatly increased risk, especially if BMI >25 kg/m²). - Females with a waist circumference ≥80 cm are at increased risk (≥88 cm = a greatly increased risk, especially if BMI >25 kg/m²).		Referral to an appropriate healthcare provider or specialist weight loss service is recommended for people with: BMI > 40 kg/m².	
Mental health/psychological distress (12, 36, 37)			
Screening requirements		Referral requirements	

All patients: Pharmacists may use a validated screening tool such as the Kessler Psychological Distress Scale (K10) ⁽³⁸⁾.	Concurrent referral to an appropriate healthcare provider is recommended for patients suspected of having a mental health disorder or in psychological distress, for example: Patients who score ≥ 20 on the K10.
Eating disorders ^(39, 40)	
Screening requirements	Referral requirements
All patients: Pharmacists may use a validated screening tool such as the SCOFF Questionnaire or InsideOut Screener	Concurrent referral to an appropriate healthcare provider is recommended for patients with or suspected of having an eating disorder.
Diabetes ^(12, 14, 15)	
Screening requirements	Referral requirements
Patients aged ≥ 40 years: - Risk of developing type 2 diabetes should be estimated every 3 years using the validated Australian type 2 Diabetes Risk Assessment Tool (AUSDRISK). All patients: - Clinical signs and symptoms of insulin resistance including acanthosis nigricans, skin tags, central obesity, hirsutism.	Not required for screening, proceed with recommended investigations/tests in Table 2.
Dyslipidaemia ^(7, 41, 42)	
Screening requirements	Referral requirements
All patients: - Family history of familial hypercholesterolaemia - Clinical signs and symptoms including tendon xanthomata, arcus cornealis before 45 years of age and xanthelasma	Concurrent referral to an appropriate health care provider is recommended for patients with a history of familial hypercholesterolemia.

Appendix 2 – Weight Management Plan

Page 1

Weight Management Plan

Plan date:

Name:		Date of birth:	
Date of commencement:			
Pharmacy details			
Pharmacist name		Phone number	
Pharmacy name and address		Opening hours	

Baseline weight and measurements

Weight:	kg	BMI:		Waist circumference:	cm
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Weight loss goal tracker:

Week beginning	Goal weight	Actual weight	BMI	Waist circumference

My lifestyle prescription

Diet and nutrition	- Referral to a dietitian for an individualised eating plan and education - Summary of nutritional advice for a balanced diet from the Australian Dietary Guidelines - more of/ increase... - less of/ limit... - Specific advice and food recommendations and strategies tailored for each individual's barriers and risks - Education and advice on interpreting nutritional labels and referral to resources to support cooking and shopping skills
Exercise and physical activity	- Enter recommendations for physical activity for patient's age and capability based on the Physical activity and exercise guidelines for all Australians: - Informal exercise e.g. building exercise into everyday activities - Formal exercise e.g., walking (moderate intensity) for 30 minutes 5 days of the week, strength building 2 days per week - individualised guidance for building up to recommendations - If required: Referral to a GP, exercise physiologist, physiotherapist or other supports for safe exercise
Other lifestyle modification strategies	- If required: Smoking cessation - Refer to Service smoking cessation program and/or other supports e.g. Quitline or GP - If required: Referral to a GP, psychologist or other clinician for mental health support - Specific advice for sleep and if required referral to a GP for management of obstructive sleep apnoea - If required: summary of advice regarding alcohol consumption based on the Australian guidelines to reduce health risks from drinking alcohol

Document version number	Date	Comments
Version 1.0	01.02.2024	
Version 1.1	11.11.2024	Administrative update.
Version 2.0	01.07.2025	Updated to reflect current clinical guidance and improved usability across sections.

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